Foreign Body Airway Obstruction

Is the Patient Conscious?

YES

Signs of Severe Airway Obstruction?
- Stridor
- Ineffective Cough
- Severe Respiratory Distress?

NO
- Reassure Patient
- Keep Patient in position of comfort
- Encourage Coughing
- Don't interfere with coughing

YES

Consider ALS Back-Up

Place patient in a position to allow gravity to assist with the procedure.

Provide 5 Back blows
then
Provide 5 Chest Thrusts

- Reassess for dislodgement after each attempt
- Repeat until object is dislodged or patient becomes unconscious

EMT

Patient Unconscious?

YES

NO

NO

Begin CPR starting with compressions.

Consider ALS Back-Up

Look in mouth for object before each ventilation.

Remove Foreign Body with finger sweep ONLY if visible.

Suction as required

- Continue High Quality CPR 30:2 (adult)
- Administer Oxygen at 15L/min

Attempt removal of the object under direct laryngoscopy
Maximum of 2 attempts.
If the object is below the vocal cords or object retrieval has failed, consider intentionally pushing the object through the right main bronchus

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Patient Care Goals

1. Removal of foreign body through the most appropriate means.
2. Early activation of ALS back-up (if indicated)

Patient Inclusion & Exclusion Criteria

Inclusion Criteria

- Mild choking (partial obstruction) where patient can breathe, cough, or speak effectively, with mild respiratory distress (e.g., coughing).
- Signs of severe airway obstruction (stridor, ineffective cough, severe respiratory distress).
- Unresponsive patient with suspected foreign body obstruction.

Exclusion Criteria

- Patients without a history of foreign body ingestion.

Key Patient Assessment Points

1. **Primary Survey**
   - Conduct a primary assessment as to Universal Care CPG to evaluate airway, breathing, circulation.
2. **Identify Airway Obstruction**
   - Look for signs of foreign body airway obstruction, including; choking, noisy breathing, stridor, inability to speak, or ineffective ventilation.
   - Assess respiratory status: Determine if the patient can breathe, cough, or speak effectively.
3. **Determine Triage Category**
   - Determine Triage Category as per Patient Triage Categories CPG.
   - All Moderate and Severe Foreign Body Airway Obstructions should be considered Red Priority.

Treatment and Interventions

1. **Consider ALS back-up**
   - Consider the requirement for ALS back-up as per Universal Care CPG.
   - Severe respiratory distress or impending respiratory failure.
   - A foreign body is observed in the airway and cannot be removed.
2. **Mild Choking** (Partial Obstruction):
   - Criteria:
     - Patient can breathe, cough, or speak effectively, with mild respiratory distress (e.g., coughing).
   - Management:
     - Encourage coughing if the patient can cough effectively.
     - Observe closely for progression to severe obstruction, using attached monitors (SpO2, ECG) to assess respiratory status and oxygenation.
     - Avoid interventions unless symptoms worsen.
     - Request ALS back-up for patients with worsening symptoms.
     - Monitor continuously: Monitor SpO2 and ECG for signs of deterioration (e.g., SpO2 <92%, increased respiratory effort).
     - Reassess frequently.
3. **Severe Choking** (Complete or Near-Complete Obstruction):
   - Criteria: Inability to speak, breathe, or cough effectively; severe respiratory distress (e.g., silent chest, cyanosis, unconsciousness).
   - Management:
     - Request ALS back-up (back-up can be cancelled if not required).
     - **Conscious Patient:**
       - Position the patient bent-over with head lower than shoulders (if safe and feasible).
       - Deliver up to 5 back blows between the shoulder blades, then up to 5 chest thrusts (avoid abdominal thrusts to prevent abdominal injury; the Heimlich maneuver is no longer recommended).

        - Repeat cycles of 5 back blows and 5 chest thrusts until the obstruction clears or the patient becomes unconscious.
        - Use monitors (SpO2, ECG) to assess response and detect deterioration.
    - **Unresponsive Patient:**
        - Begin CPR if the patient becomes unresponsive, starting with compressions.
        - Look in mouth for object before ventilation and remove with finger sweep – only if object is visible.
        - Perform direct laryngoscopy and use Magill's forceps to remove the foreign body as soon as possible after the patient becomes unresponsive.
            - A maximum of 2 attempts at removal under direct laryngoscopy are permitted.
        - Consider endotracheal intubation, initially placing the tube deep, then pulling back slightly to manoeuvre the foreign object into the bronchus, allowing airflow through the other side, if available airway skills are evident.
    - Administer oxygen at 15L/min via non-rebreather or Bag-Valve-Mask.

4. **Patient Monitoring & Reassessment**
    - Frequently Reassess the patient as per Universal Care CPG.
        - Specific Patient Reassessment Points:
            - Reassess after each intervention for airway patency or worsening symptoms.
            - Continuously reassess the patient's airway status, breathing, and response to treatment using attached monitors (SpO2, EtCO2, ECG).

Patient Safety Considerations

- Patient populations at risk
    - Young children, older adults and people with intellectual disabilities, as they are more prone to FBAO. Adjust assessment, monitoring and interventions for these groups; consider potential communication or compliance challenges.

Clinical Insights & Additional Information

- Esophageal obstruction
    - If a foreign object is suspected in the esophagus (e.g., discomfort or difficulty swallowing saliva), offer small amounts of liquid to assist passage, but transport to the closest appropriate hospital if unsuccessful. Avoid airway interventions unless airway compromise is present.
- Foreign body ingestion
    - For ingested objects in young children, assess for excessive saliva, difficulty swallowing, or abdominal pain. Transport urgently to the closest most appropriate surgical-capable hospital if the object is hazardous (e.g., batteries, magnets, sharp items, especially button batteries due to severe injury risk).

Documentation

- Complete a comprehensive electronic Patient Care Record (ePCR), capturing the following elements to support continuity of care. Including:
    - Identification: Full patient name, age, and gender.
    - Mechanism / Medical Complaint: Presenting symptoms (e.g., choking, stridor), initial airway status.
    - Injuries / Information: Relevant history (e.g., ingestion, trauma, substance use).
    - Signs:
        - Full vital signs including pulse, respiratory rate, SpO2, EtCO2.
        - Two sets of vital signs unless critical patient condition precludes.
        - Airway assessment findings (e.g., patency, obstruction).
    - Treatment:
        - Airway management steps and oxygen administration.
        - All interventions: back blows, chest thrusts, CPR, foreign body removal attempts, and response.
        - Justification if interventions were withheld.

- Allergies: Documented allergies and drug interactions.
- Medications: All pre-existing medications and any administered in the field.
- Background: Relevant medical history, comorbidities, and recent procedures.
- Other:
  - Communication with Telemedicine (time and instructions).
  - Response to interventions.
  - Consideration of reversible causes and actions taken.

Key Performance Indicators

1. Document the initial airway assessment findings for all patients with FBAO.
2. Provide an initial intervention (e.g., back blows or chest thrusts) within 1 minute of identification.

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